



Guideline

Take Home Naloxone

Scope

Site	Service/Department/Unit	Disciplines
SCGH OPH	Emergency Department Pain management clinic	Medical, Nursing, Pharmacy Medical, Nursing, Pharmacy

Introduction

Opioids are the leading cause of unintentional drug-related deaths in Australia. Naloxone is a medication capable of preventing death from opioid overdose if administered promptly. It is recommended to provide take-home naloxone (THN) upon discharge for patients at risk of opioid overdose or individuals likely to witness such an event. This practice ensures timely and equitable access to this life-saving medicine for those in need.^[1]

Definitions

Opioids	A broad category encompassing natural or synthetic substances that interact with opioid receptors. They are often utilised for their analgesic, gastrointestinal, and respiratory effects. ^[2] Opioids may be prescribed for therapeutic purposes or used recreationally.
Naloxone	Competitive antagonist at opioid receptors which may be administered to reverse the effects of an opioid overdose. ^[2]
Support Person	A family member, next of kin, carer or friend who is willing and able to assist the patient in using naloxone if required.

Supplying Health Practitioner

Take-home naloxone (THN) may only be provided for use upon discharge by Medical Practitioners, Alcohol and Other Drug (AOD) Nursing staff, and nursing staff who have completed formal THN education through the Mental Health Commission or an approved trainer at SCGOPHCG. Health practitioners must comply with legal requirements for supply and maintain accurate records, including the date of supply, medicine, and quantity supplied, in the patient's clinical notes. Pharmacy is not required to be directly involved in the supply process unless the treating team requires clinical guidance. For further information about supply, refer to the [SCGOPHCG Structured Administration and Supply Arrangement \(SASA\): Take Home Naloxone \(THN\)](#)

If at any time trained staff members are not available, the patient is to be advised to go to a community drug and alcohol service or a General Practitioner (GP) to obtain a prescription to purchase the prefilled syringe from a community pharmacy. Over the counter purchase without a prescription is more costly but an option.

For more information or referrals, the AOD team can be contacted between 7 am and 3 pm, Monday to Friday (excluding public holidays).

Patient Assessment

Patients may be deemed to be at risk of future opioid overdose if they meet ≥ 1 of the following criteria:

- Have previously experienced an opioid overdose
- Have a suspected or confirmed history of heroin or non-medical opioid use, or use of other illicit drugs in an unregulated market where the drug supply may contain opioids without their knowledge (e.g. ketamine or cocaine)
- Taking an opioid concurrently with a benzodiazepine, pregabalin, gabapentin, an antidepressant, an antipsychotic or other CNS depressants
- Are opioid naive and have been prescribed a quantity or strength of opioids that may represent a risk
- Have decreased tolerance due to a recent break or change in opioid use (e.g. have been released from custodial settings, recent detox or have undertaken a drug withdrawal program)
- Current opioid use with chronic medical conditions (e.g. COPD, asthma, sleep apnoea)
- Use of more than 50 mg oral morphine (or equivalent) per day or long-acting opioids

Inclusion Criteria

Naloxone may be supplied to either the patient or the patient's support person if they meet all of the following criteria:

- Aged 16 years or over AND
- At risk of a future opioid overdose or are likely to witness an opioid overdose AND
- Can demonstrate sufficient knowledge on the safe use of naloxone

Exclusion Criteria

The patient or patient's support person should not receive naloxone on discharge if they meet one or more of the following criteria:

- Under the age of 16 years AND/OR
- Patient has a known allergy to naloxone AND/OR
- Unable to demonstrate sufficient knowledge on the safe use of naloxone

Available Products

Please check [Formulary One](#) for inpatient locations.

No more than two products may be supplied at one time, which can consist of either two units of the same product or one unit of each of the following products:

- Prenoxad: Naloxone 1 mg/mL prefilled syringe (2 mL / 5 doses)
- Nyxoid: Naloxone 1.8 mg nasal spray (2 pack / 2 doses)

Dose and Administration

Prenoxad: Naloxone 1 mg/mL prefilled syringe (2 mL / 5 doses)

- Give 0.4 mL via IM injection (to first black line on syringe) into the outer thigh or upper arm muscle, repeat every 2-3 minutes until ambulance arrives or patient regains consciousness

Nyxoid: Naloxone 1.8 mg/actuation nasal spray (2 pack / 2 doses):

- 1 spray (1.8 mg) into one nostril; if needed, a second dose (using a new device) may be sprayed into the other nostril after 2–3 minutes. Additional doses may be given if needed, until adequate emergency medical care is available.

Counselling

Prenoxad and Nyxoid may only be supplied to the patient or the patient's support person if the appropriate counselling has been provided. This involves discussing risk factors for opioid overdose and how to recognise and respond to an opioid overdose. Ensure specific directions on using Prenoxad and/or Nyxoid are discussed and understood. A useful resource to assist in counselling is the [WA Mental Health Commission Recognise and Respond Leaflet](#). This leaflet will be available in a printed version next to the naloxone products held on imprest.

If trained staff members are not available at any time, advise the patient that they can obtain free stock from other services participating in the Take Home Naloxone Program, such as Next Step, Hepatitis WA, Peer Based Harm Reduction WA and some community pharmacies. For a full list of registered suppliers see [Take Home Naloxone Registered Suppliers](#).



Nyxoid® (an intranasal naloxone preparation) is available free of charge from many pharmacies in WA, and a number of other sites. It may also be prescribed & is included in the PBS.

This QR code links to pharmacies where THN is available

Compliance, monitoring and evaluation

Compliance and monitoring of this guideline will be conducted by the Alcohol and Other Drugs Service (SCGH) and Pharmacy (OPH).

Acknowledgments

We acknowledge the following site endorsed guidelines used in preparation of this guideline:

- Fiona Stanley Hospital – Naloxone prefilled take-home syringes (prenoxad) and nasal delivery device (Nyxoid) (Clinical Guideline FSFH-HW-GUI-0029)
- Royal Perth Hospital – Prenoxad and Nyxoid Supply via the Take Home Naloxone Pilot SOP August, 2022

Related Documents

Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)
- [Health Services Act 2016](#)

Authorising Policy and Standard/s:

- [SCGOPHCG Medicines Management Policy and Procedure](#)
- [SCGOPHCG High Risk Medication Policy](#)
- [Mandatory Policy MP0077/18 Statewide Medicines Formulary Policy](#)

- NSQHS Standards: 1,2, 4,5

Standards, procedures, guidelines:

- [SCGOPHCG Structured Administration and Supply Arrangement \(SASA\): Take Home Naloxone \(THN\)](#)

References

1. Government of Western Australia Mental Health Commission. Take-home naloxone [Internet]. Western Australia: Mental Health Commission; [cited 2024 March 14]. Available from: <https://www.mhc.wa.gov.au/reports-and-resources/resources/health-professional-resources/take-home-naloxone/>
2. Australian Medicines Handbook. [Internet]. Adelaide (South Australia): Australian Medicines Handbook; 2024 [cited 2024 March 14] Available from: <https://amhonline-amh-net-au.kelibresources.health.wa.gov.au>

Authorisation & Version Control

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Standards Applicable	Insert relevant NSQHS Standards (Version 2): 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.